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County: los-angeles

Division: Civil Law and Motion

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Case Number: 25STCV36834 Hearing Date: September 22, 2026 Dept: 14

SUPERIOR COURT OF THE STATE OF
CALIFORNIA

FOR THE COUNTY OF LOS ANGELES

DEPARTMENT 14

CLAUDIO SEBASTIA and MARIANA MARINO AGUIRRE,

Plaintiffs,

v.

7-ELEVEN, INC., et al.

Defendants.

Case No. 25STCV36834

Hearing Date: September 22, 2026

Time:

9:00 a.m.

¿ [TENTATIVE] ORDER

RE:¿

PLAINTIFFS' MOTION FOR TRIAL PREFERENCE

I.

Background

On December 16, 2025, Plaintiffs Claudio Sebastia and Mariana Aguirre filed their complaint for personal injury alleging Mr. Sebastia, who is 66 years old, developed mesothelioma from direct exposure to asbestos-containing products from his work as a mechanic from 1982-1986. On April 10, 2026, the court denied Plaintiffs' first motion for trial setting preference under Code Civ. Proc., § 36, subds. (d) and (e). On August 25, 2026, Plaintiffs filed a renewed motion for trial preference. They contended that that there is substantial medical doubt that Mr. Sebastia will survive beyond another six months and that the interests of justice are best served by granting his motion.

On September 9, 2026, Defendants American Honda Motor Co., Inc. ("Honda"), Nissan North America, Inc. ("Nissan"), Ford Motor Company ("Ford"), and Volvo Car USA LLC ("Volvo") opposed the motion for trial preference. On the same date, Defendants Pneumo Abex, LLC ("Pneumo"), Akebono Brake Corporation, 7-Eleven, Inc., and Tracy Industries, Inc. joined Ford's opposition. Defendants argue that Plaintiffs failed to demonstrate by clear and convincing medical documentation that there is substantial doubt of Mr. Sebastia's survival beyond six months and that granting trial preference would not serve the interests of justice.

On September 15, 2026, Plaintiffs replied, arguing that their new evidence justifies the granting of trial preference.

The court finds that Plaintiffs failed to satisfy their burden to show they are entitled to trial preference under Code of Civil Procedure section 36, subdivisions (d) and (e). The evidence does not clearly and convincingly support a finding that there is significant medical doubt of Mr. Sebastia's survival beyond six months. Additionally, the evidence fails to support a finding that the interests of justice would be served by granting trial preference. The court denies Plaintiffs' motion for trial preference.

II.

Discussion

A.

Legal Standard

The court has discretion to grant a motion for trial preference accompanied by clear and convincing medical documentation concluding that one of the parties suffers from an illness or condition raising substantial medical doubt of survival of that party beyond six months and satisfying the court that the interests of justice will be served by granting the preference. (Code Civ. Proc., § 36, subd. (d).) The court may also grant a motion for preference in its discretion if it is supported by a showing that satisfies the court that the interests of justice will be served by granting the preference. (Id., § 36, subd. (e).)

“Upon the granting of such a motion for preference, the court shall set the matter for trial not more than 120 days from that date and there shall be no continuance beyond 120 days from the granting of the motion for preference except for physical disability of a party or a party’s attorney, or upon a showing of good cause stated in the record.” (Id., § 36, subd. (f).) “Any continuance shall be for no more than 15 days and no more than one continuance for physical disability may be granted to any party.” (Ibid.)

B.

Analysis

This

is Plaintiffs’ second motion for trial preference. Mr. Sebastia is 66 years old; in October 2025, he was diagnosed with malignant mesothelioma. (Presser Decl. at ¶¶6-7.) Mr. Sebastia was hospitalized between September 26, 2025 and October 19, 2025, requiring chest tube placement to treat pleural fluid buildup and pneumothorax. (Id. at ¶8.) During his hospitalization, a CT scan revealed a soft tissue density in his chest that displaced his esophagus to the left. (Id.) After his discharge, Mr. Sebastia’s doctors informed him that mesothelioma at his stage was not curable and that he was not a candidate for surgery. (Id. at ¶9.) Mr. Sebastia then began receiving palliative care consisting of chemotherapy and immunotherapy. (Id. at ¶¶9-11.) At a January 28, 2026 telemedicine appointment with his physician, Dr. Eric Presser, Mr. Sebastia reported 10 pounds of weight loss since his diagnosis, lack of

appetite, fatigue from treatment, insomnia, chest pain, shortness of breath, and cough. (Id. at ¶12.) In January 2026, Dr. Presser opined based on this visit and his review of medical records that there was substantial medical doubt of Mr. Sebastia's survival beyond six months. (Id.) In March 2026, new CT scans revealed that the mass in his chest had grown to 6.6 x 3.4 cm, a small right pleural effusion, pleural thickening, and prominent mediastinal nodes. (Presser Decl. at ¶13, Exh. B.) Mr. Sebastia's medical records from this time show that his breathing was stable, had no musculoskeletal pain, and reported no headache or dizziness. (Rosenthal Decl., Exh. 6.) In July 2026, a PET/CT scan showed "increased streaky and bandlike opacities along the right to mid lower lung zone" and a small right pleural effusion. (Presser Decl., Exh. C.) A PET scan from late July 2026 showed increased FDG uptake in Mr. Sebastia's chest. (Id., Exh. D.) Dr. Presser opines that the PET scan results show that the masses in Mr. Sebastia's chest had increased, indicating increased metabolic activity and disease progression. (Id. at ¶¶15-16.)

Dr.

Presser opines that Mr. Sebastia's disease has progressed since the previous declaration was submitted and there is a substantial doubt he will survive beyond six months:

"17. It is clear

from the radiologic evidence and the physical and cognitive symptoms of his disease progression that his mesothelioma has continued to progress and will continue to do so. He remains at risk of it spreading to his heart, bowels, or lungs, which would likely lead to a rapid decline and death in a matter of weeks. Mr. Sebastia is already suffering increasing shortness of breath and fatigue from his mesothelioma and is at high-risk of rapid decline from infection or cancer progression. The July 2026 PET-CT findings provide additional objective support for my opinion that Mr. Sebastia's disease is progressing and will continue to progress, leading to further decline in the coming months.

18. Further, due

to Mr. Sebastia's weakened condition and compromised pulmonary functioning, he is particularly susceptible to further complications, including breathing problems, pneumonia, bronchitis, colds, flu, and COVID-19. In his weakened state, any of these conditions can be life-threatening.

19. Based on my

education, training and experience, as well as my knowledge of Mr. Sebastia's diagnosis and observation of his disease progression, it is my opinion, within a reasonable degree of medical certainty, that he will continue to experience increasing pain, shortness of breath and weight loss; that his symptoms and condition will worsen; and that he is now, and will remain, at high risk for rapidly-developing, life-threatening infections and other complications likely to arise from the progression of his disease. Mr. Sebastia is terminally ill and his prognosis is very dire.

20. Based on the updated July 2026 PET-CT showing increased FDG uptake in pleural based masses in the right hemithorax and markedly increased FDG uptake in right lateral abdominal wall masses, it is my further opinion that Mr. Sebastia's disease has shown objective radiographic progression since the original declaration.

21. Therefore, based on all of the above supporting information, including my education, experience treating mesothelioma patients, and my knowledge of Mr. Sebastia's personal medical history, the fact that he was diagnosed with mesothelioma in October 2025, the presence of cancer in his lung lining and abdomen and several lymph nodes, the results of his most recent scan showing significant progression of the cancer and the deterioration he has experienced since my last evaluation, it is my professional opinion that there is substantial medical doubt of Mr. Sebastia's survival beyond another six months."

(Rosenthal Decl., Exh. 3 at ¶¶17-21.) Dr. Presser now opines that Mr. Sebastia's PET/CT scans and worsening symptoms show that he is declining and that there is substantial medical doubt of his survival beyond another six months. (Id. at p.21.)

Mr. Sebastia declares that his condition has worsened since January 2026 and that he requires hydrocodone twice a day to manage his pain. (Sebastia Decl. at ¶5.) He reports feeling dizzy and that he spends much of the day lying down or sitting. (Id. at ¶6.) Mr. Sebastia reports that he requires help showering because he fears falling and that he can no longer lift his tools, perform chores, or drive. (Id. at ¶¶7-10.)

Plaintiffs'

counsel, Deborah Rosenthal, declares that in her 20 years of litigating asbestos cases, she has observed stable clients decline and pass away in a matter of weeks. (Rosenthal Decl. at ¶10.) Ms. Rosenthal argues that dying mesothelioma patients, such as Mr. Sebastia, require preferential trial dates within one year of diagnosis. (Id.) Ms. Rosenthal further argues that the interests of justice would be served by a preferential trial date because Defendants and their counsel have had ample opportunity to conduct discovery, Plaintiffs have produced all of Mr. Sebastia's medical and billing records, and Mr. Sebastia has already been deposed. (Id. at ¶11.) Ms. Rosenthal declares that Plaintiffs will submit to supplementing discovery responses as needed, agree to modifications of pre-trial deadlines, and agree to shortened notice periods for discovery responses and motions. (Id. at ¶12.)

In opposition, Defendants argue that

Mr. Sebastia's new medical records fail to support Dr. Presser's opinion that Mr. Sebastia is not likely to survive beyond six months. (Ford Opp. at pp. 5-8, Honda opp. at pp. 3-6, Volvo opp. at pp. 3-5.) Specifically, the new records documenting the July 2026 CT scan and Dr. Presser's updated declaration fail to show by clear and convincing evidence that there is substantial medical doubt of Mr. Sebastia's survival beyond six months.

In reply, Plaintiffs argue that the

new medical records support Dr. Presser's opinion that there is substantial medical doubt of Mr. Sebastia's survival beyond six months. (Reply at pp. 4-6 [“Plaintiff is not required to show that every imaging report independently proves the same fact. As Dr. Presser explained, the March CT and July x-ray provide additional evidence of persistent, growing disease. This growth shows cancer progression, and as Dr. Presser explains, as the cancer progresses, Mr. Sebastia's symptoms worsen, the attendant risks increase, and the likelihood of his continued survival diminishes.”].)

Plaintiffs

were required to show by clear and convincing medical documentation that there is substantial medical doubt of Mr. Sebastia's beyond six months and that the interests of justice would be served by granting trial preference. (Code Civ. Proc., § 36, subd. (d).) Although Plaintiffs' counsel and Dr. Presser opine that Mr. Sebastia will not survive longer than six months, the record does not support this with clear and convincing evidence. Mr. Sebastia's updated medical records from March 2026 and July 2026 show that he was doing well overall. While the updated CT and PET scans show progression of Mr. Sebastia's disease, the scans show that the

masses in his chest have somewhat increased. Mr. Sebastia's declaration shows that he can no longer perform household tasks but is insufficient to show that there is substantial medical doubt of his survival beyond six months. The evidence does not support Dr. Presser's opinion regarding Mr. Sebastia's survival beyond six months. The evidence is insufficient to meet the clear and convincing standard required by Code of Civil Procedure section 36, subdivision (d).

This denial of a preferential trial setting is without prejudice to a motion based on future changed circumstances.

III.

Conclusion

Plaintiffs' motion for trial setting preference is denied.

Plaintiffs are ordered to give notice.